

Sample Hospital

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DISCHARGE SUMMARY

Patient: Poe, Sarah K.	DOB: 07/30/1949	MRN: SH-2025-091204
Admission Date: 02/11/2025	Discharge Date: 02/15/2025	Attending: Dr. Jane Smith, MD
Service: Internal Medicine	Room: 5-East, Bed 520A	Insurance: Sample Insurance Company

CHIEF COMPLAINT

Productive cough, fever, and shortness of breath for four days.

HISTORY OF PRESENT ILLNESS

Mrs. Poe is a 75-year-old female with a history of COPD and type 2 diabetes who presented with a four-day history of productive cough with yellow-green sputum, subjective fevers, and progressive dyspnea. On arrival she was febrile to 101.8°F, tachypneic, and hypoxic with an oxygen saturation of 88% on room air. Chest radiograph demonstrated a right lower lobe consolidation consistent with community-acquired pneumonia.

HOSPITAL COURSE

Patient was admitted and started on IV ceftriaxone and azithromycin per community-acquired pneumonia guidelines, with supplemental oxygen and scheduled bronchodilators. Blood cultures showed no growth. She defervesced by hospital day 2 and was weaned to room air by day 3 with oxygen saturation of 94%. Transitioned to oral cefuroxime to complete the course. Diabetes remained well controlled on her home regimen during admission.

DISCHARGE DIAGNOSES

ICD-10	Description	Type
J18.1	Lobar pneumonia, unspecified organism	Primary
J44.1	COPD with acute exacerbation	Secondary
E11.9	Type 2 diabetes mellitus without complications	Secondary

MEDICATIONS AT DISCHARGE

Medication	Dose	Route	Frequency	Duration
Cefuroxime	500 mg	PO	BID	5 days to complete course
Albuterol/Ipratropium	1 neb	INH	Q6H PRN	Ongoing
Prednisone	40 mg	PO	Daily	5-day taper
Metformin	1000 mg	PO	BID	Home medication
Guaifenesin	600 mg	PO	BID PRN	Cough

FOLLOW-UP INSTRUCTIONS

1. Primary care follow-up in 7 days (appointment scheduled 02/22/2025)
2. Complete full antibiotic course even if feeling better
3. Repeat chest X-ray in 6 weeks to confirm resolution
4. Return to ED if: fever returns, worsening shortness of breath, or chest pain

CONDITION AT DISCHARGE

Stable. Improved. Oxygen saturation 94% on room air.

Jane Smith, MD

Department of Internal Medicine

Electronically signed 02/15/2025 15:10 CST

SYNTHETIC DATA — FOR DEMONSTRATION PURPOSES ONLY — NO REAL PATIENT INFORMATION